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FreeMedicareTools.com

Medicare 101

A plain-English guide for people turning 65 and the families who help them

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GUARDIAN Benefits Group

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We do not offer every plan available in your area. Currently we represent 8 organizations which offer 50 products in your area. Please contact Medicare.gov or 1-800-MEDICARE to get information on all of your options.

Counts are for Clark County, NV, 2027 plan year. 1-800-MEDICARE is available 24 hours a day, 7 days a week (TTY 1-877-486-2048).

Not affiliated with or endorsed by the U.S. government or the federal Medicare program. Licensed in NV, CA, TX, HI, UT.

The questions we'll answer today

1. What is Medicare?
2. Who can get Medicare?
3. What if I keep working past 65?
4. What do Parts A and B cover?
5. What doesn't Original Medicare cover?
6. What are my two coverage paths?
7. What does Medicare cost?
8. When should I enroll?
9. How do the paths compare, and when can I change?
10. How can I save, and where can I get help?

What Medicare is and what it isn't

Medicare IS

- Federal health insurance for 65+ and some younger people
- Individual: one person at a time
- Partly paid for by taxes from your paychecks

Medicare is NOT

- Free: you'll still have costs
- A family plan: spouses enroll separately
- Social Security or Medicaid

Medicaid is a state program for people with limited income. Some people qualify for both. That's called being “dual eligible.”

Who can get Medicare?

U.S. citizens, or permanent residents who've lived here 5 years in a row, who also meet one of these:

65+

Age 65 or older

Under 65

With a disability, after 24 months of Social Security disability (SSDI)

ESRD

Any age, with kidney failure that needs dialysis or a transplant

ALS

Any age, with Lou Gehrig's disease. Medicare starts when SSDI does

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Try it: Still Working Past 65?
FreeMedicareTools.com/tools/working-past-65

Still working at 65?
Your employer's size matters

20
employees
is the number to know

- 20 or more employees: you can usually delay Part B with no penalty while you have job-based coverage
- Fewer than 20: Medicare usually pays first, so plan on Parts A and B at 65
- Have an HSA? Stop contributing before Medicare starts. Part A can backdate up to 6 months
- Ask HR whether your drug coverage is “creditable,” and keep that letter

Part A: hospital insurance

- Inpatient hospital stays
- Skilled nursing care after a qualifying stay
- Hospice care
- Some home health care

Premium-free

for most people, if you or your spouse worked and paid Medicare taxes for about 10 years

Doctors' services in the hospital fall under Part B.

Part B: medical insurance

20%

is what you usually pay for covered care, after the yearly deductible

Part B has a monthly premium. Costs are coming up.

Part B helps pay for

- Doctor visits, including in the hospital
- Outpatient care, tests and X-rays
- Preventive care and screenings
- Equipment like walkers and oxygen
- Ambulance and some home health

What Original Medicare doesn't cover

- x Most dental care
- x Eye exams for glasses
- x Hearing aids and exams
- x Long-term custodial care
- x Care when you travel abroad, in most cases
- ! **No yearly cap on your out-of-pocket costs**

That's why most people add more coverage. There are two main ways to do it.

Two paths to fill the gaps

Path 1: Original Medicare

Part A · hospital

Part B · medical

+ Part D · drug plan

+ Medigap · optional, fills gaps

or

Path 2: Medicare Advantage

Part C: one private plan that bundles A + B, and usually D

Uses a network, has a yearly out-of-pocket limit, and may add extras that vary by plan

Part C: Medicare Advantage

Private companies approved by Medicare offer these plans. You still pay your Part B premium.

01 Covers almost everything Parts A and B cover (hospice is covered by Original Medicare)

02 Most include Part D drug coverage

03 Usually uses a network (HMO or PPO)

04 Has a yearly limit on out-of-pocket costs

05 Some add extras like dental, vision or hearing; these vary by plan

Part D: prescription drug coverage

- Sold by private companies approved by Medicare
- Get it as a stand-alone plan with Original Medicare, or inside a Medicare Advantage plan
- Each plan has its own drug list and pharmacies
- Going without creditable drug coverage can mean a penalty for as long as you have Part D

$2,100

yearly cap on what you pay out-of-pocket for covered drugs in 2026

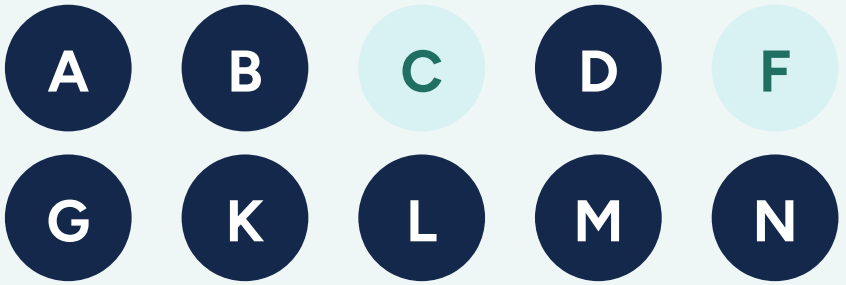
2026 figure. The cap rises to $2,400 in 2027.

Try it: Medigap Plan Letter Guide
FreeMedicareTools.com/tools/med-supp-compare

Medigap (Medicare Supplement)

- Sold by private companies; works only with Original Medicare
- Helps pay your share: deductibles, the 20% and more
- See any U.S. doctor who takes Medicare
- Doesn't include drugs, so pair it with Part D

Plans are standardized by letter



Same letter, same basic benefits from any company in most states. Prices differ. (Massachusetts, Minnesota and Wisconsin differ.)

Plans C and F: only if you were first eligible for Medicare before Jan 1, 2020

What Medicare costs in 2026

2026 figures. 2027 amounts are usually announced in the fall.

$202.90

Part B standard monthly premium

$283

Part B yearly deductible

$1,736

Part A hospital deductible, per benefit period

Higher income? You may pay more for Parts B and D (called IRMAA). In 2026 it starts above $109,000 single or $218,000 joint, based on your 2024 taxes.

Try it: IEP Timeline Calculator
FreeMedicareTools.com/tools/iep-timeline

When to enroll: your 7-month window

3 months before	2 months before	1 month before	**Your 65th birthday month**	1 month after	2 months after	3 months after

Initial Enrollment Period (IEP). Signing up in the 3 months before avoids a gap.

General Enrollment Period

Jan 1 – Mar 31 each year if you missed your IEP. Coverage starts the month after you sign up; penalties may apply.

Medigap Open Enrollment

6 months, starting the month you're 65+ and have Part B. During this window, companies can't deny you or charge more because of health problems.

Try it: Late Enrollment Penalty Estimator
FreeMedicareTools.com/tools/penalty-estimator

Special enrollment and late penalties

Special Enrollment Period

- Job-based coverage ending (yours or your spouse's, from current work)? You get 8 months to sign up for Part B. COBRA and retiree coverage don't extend it.
- About 2 months to join a drug plan
- Moving and other life changes can open a window too

10% Part B: 10% added to your premium for each full 12 months you could have had Part B but didn't, usually for as long as you have Part B

1% Part D: 1% of a national base amount for each month without creditable drug coverage, for as long as you have Part D

Common coverage combinations

A B	Original Medicare alone: the most gaps
A B D	Adds drug coverage
A B D Medigap	More predictable costs; any doctor who takes Medicare
Medicare Advantage (Part C)	One plan, with a network and copays

Some people also keep employer or retiree coverage alongside Medicare.

Medigap vs. Medicare Advantage

	Original + Medigap + Part D	Medicare Advantage
Doctors	Any U.S. doctor taking Medicare	Usually a plan network
Monthly cost	Medigap and Part D premiums, plus Part B	Plan premium varies, plus Part B
When you get care	Usually lower and more predictable (depends on the plan letter)	Copays as you go, up to a yearly limit
Drug coverage	Separate Part D plan	Usually included
Changing later	May need to answer health questions	Can switch each fall

Try it: AEP Annual Review Checklist
FreeMedicareTools.com/tools/aep-checklist

Changing coverage: two yearly windows

Annual Enrollment Period (AEP)

Oct 15 – Dec 7

Anyone with Medicare can switch plans, or join, switch or drop a drug plan. Changes start Jan 1.

Medicare Advantage Open Enrollment

Jan 1 – Mar 31

Already in Medicare Advantage? Make one switch, or go back to Original Medicare.

You can apply for Medigap anytime, but outside your Medigap Open Enrollment Period you may face health questions.

Help paying for Medicare

Extra Help (LIS)	Lowers Part D drug costs for people with limited income and savings. Apply at SSA.gov.
Medicare Savings Programs	Your state may pay your Part B premium, and sometimes more.
Nevada Medicaid	Covers more of your costs if you qualify. Apply through Access Nevada (Nevada Division of Social Services). Outside Nevada? Contact your state Medicaid office.

Qualify for a Medicare Savings Program? You automatically get Extra Help too.

Free help from Medicare, Social Security and your SHIP

Medicare.gov

Plans, costs and coverage rules

1-800-MEDICARE

1-800-633-4227
TTY 1-877-486-2048
24/7, except some federal holidays

Nevada SHIP (MAP)

Free counseling: 1-800-307-4444
Outside Nevada? Visit shiphelp.org

Social Security

SSA.gov or 1-800-772-1213
(TTY 1-800-325-0778)

These are free government and nonprofit resources you can use anytime.

Your next steps

Start 3 to 6 months before your 65th birthday month.

1. Write down your Initial Enrollment Period dates
2. Still working? Ask HR about group size and creditable coverage
3. List your doctors, prescriptions and pharmacies
4. Learn the two paths: Original Medicare (with Part D and optional Medigap) or Medicare Advantage
5. Sign up for Parts A and B through Social Security
6. Want one-on-one help later? Book a free consult

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Explore the free tools:
FreeMedicareTools.com/tools

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If you'd like one-on-one help later, book a free consult

Book online: https://calendar.app.google/CEnDfzoQbNBMAduz9

Or call a licensed insurance agent: **702-627-2605 (TTY 711)**

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Presenter instructions (skipped — not shown)

1. Open the deck in Google Slides.
2. Click Slideshow, then Presenter view, so your notes open in a separate window.
3. In Google Meet, click Present now, then "A tab" or "A window," and share the slideshow window, not the presenter view.
4. Only one monitor? Print the notes or open them on a phone or tablet: File > Print settings and preview > 1 slide with notes.
5. Do a test run first in a solo Meet.
6. At the start, paste the disclaimer into Meet chat (the TPMO statement, the federal line and "Licensed in NV, CA, TX, HI, UT"; full text in the slide 1 notes), then read the opening disclaimer on slide 1.
7. Recording: the default is not to record. If you do record, every attendee must say yes out loud first, and you restate that consent once the recording starts (script on slide 1). If anyone declines, don't record.
8. Update the 2026 figures on slides 11 and 13 (and the matching script lines on 11, 13 and 19) every fall, and re-pull the 8 organizations / 50 products counts each plan year.
9. Today is education only. No plan-specific benefits, premiums, comparisons or applications. Before any follow-up that discusses plans, record an SOA (in writing if in person) per 42 CFR 422.2264(c)(3)(i), plus any stricter FMO or carrier rule (some still require 48 hours; confirm). If you collect SOAs or contact info at the event, it must be optional. Before scheduling, confirm with Pinnacle/carriers whether this agent-hosted event must be reported or filed in their portals, and their SOA rules.
10. Tool plugs (slides 1, 5, 12–15, 18, 21, 22) point to FreeMedicareTools.com, which Guardian runs. Keep them educational and never mention the site on slide 20.